

Making the Most of Clinical Encounters

Prevention of Child Abuse and Maltreatment



Iram J. Ashraf, MD, Alicia R. Pekarsky, MD, JoAnne E. Race, MS,
Ann S. Botash, MD*

KEYWORDS

• Parenting • Child maltreatment • Neglect • Sexual abuse • Sentinel injuries

KEY POINTS

- Risk factors for child abuse vary depending on age, developmental factors, social context, and type of abuse.
- Support for parents is critical for prevention efforts.
- Selecting appropriate resources, based on risks and type of abuse, requires understanding of local programs.
- Recognition of physical findings, particularly sentinel injuries, is necessary to prevent further abuse.
- Sexual abuse prevention requires recognition of subtle signs, including signs of trafficking and peer victimization.

INTRODUCTION

Childhood experiences occur within the context of family and community and this context may include child abuse, maltreatment, and neglect. Child abuse is a public health concern with great costs to not only children and families, but also to society. Abused children suffer physical injuries, psychological problems, learning difficulties, a spectrum of neurologic sequelae, and death. Abused children grow up to be adults at risk for depression, suicide, substance abuse, and criminal activity and are at higher risk for abusing children themselves.¹

Gaps in physician knowledge and skills and discomfort in addressing child maltreatment impede prevention efforts.² In addition, the US Preventive Services Task Force, the American Academy of Family Practitioners, and the American Academy of

State University of New York Upstate Medical University, Upstate Golisano Children's Hospital, Department of Pediatrics, 750 East Adams Street, Syracuse, NY 13210, USA

* Corresponding author.

E-mail address: BOTASHA@UPSTATE.EDU

Twitter: @Botasha (A.S.B.)

Pediatr Clin N Am 67 (2020) 481–498

<https://doi.org/10.1016/j.pcl.2020.02.004>

0031-3955/20/© 2020 Elsevier Inc. All rights reserved.

pediatric.theclinics.com

Pediatrics (AAP) have no recommendations for primary care interventions to prevent child maltreatment. However, the US Preventive Services Task Force report notes that primary care physicians, including pediatricians and others, are uniquely positioned to identify child maltreatment.³ The AAP strongly recommends clinician involvement in preventing child maltreatment and provides guidance and information on risk factors, protective factors, and clinical management.²

The authors used the three-tiered public health framework for prevention (primary, secondary, and tertiary) to guide strategies for clinical encounters.⁴ In the current paper, we discuss how providers can identify, address, and prevent child maltreatment through parent education and use appropriate programs. This discussion includes a review of early intervention resources, summaries of strategies, and an overview of prevention approaches for neglect, physical abuse, head trauma, sexual abuse, and peer victimization.

PUBLIC HEALTH PREVENTION FRAMEWORK

The public health prevention framework aids in categorizing programs.

- Primary prevention programs are population-based and engage the community. These may include home visits from a nurse and/or provision of parenting information and education. Strategies focus on addressing risk factors. Primary prevention offers the best opportunity to reduce long-term sequelae of child maltreatment by preventing trauma to the developing brain during the first 3 years of life.⁵
- Secondary prevention programs target families with specific risk factors for child maltreatment.
- Tertiary prevention programs are designed to prevent maltreatment recurrence and adverse outcomes from abusive experiences.

Tertiary prevention strategies can be provided during ongoing pediatric health care for children who have been maltreated.⁶ Maximizing health care and services for maltreated children after they return to their biologic homes from foster or kinship care can be offered through anticipatory guidance. Important prevention considerations for postabuse follow-up include assessment of risks for recidivism in families; close monitoring of developmental milestones, especially in children where head trauma occurred; monitoring of behavioral changes caused by family and possible cultural adjustments; and medical concerns related to the abusive injuries.⁶

IDENTIFYING RISK FACTORS FOR ABUSE AND MALTREATMENT

Parent or caregiver characteristics and psychological functioning, characteristics of the child, and the social context within which the family unit lives influence parenting methods.⁷ Within these components are several risk factors for the abuse and maltreatment of children. Parental stress is a common denominator.

Poor Parenting Skills

Parents who have inadequate knowledge of normal and expected child development are at risk for demonstrating poor parenting skills and this increases the risk for maltreatment.^{5,8} Parental knowledge deficits may be caused by a lack of a good parenting model, lack of previous exposure to children, the parent's own developmental delays or emotional issues, and a lack of knowledge regarding normal child development, or other reasons. For example, a parent who believes a 1 year old should be capable of understanding the consequences of touching a hot mug might leave the mug within reach of a child. Some parents believe in the success of corporal punishment and lack knowledge and skills for effective disciplinary strategies.⁷ Those

who use harsh, inattentive, or inconsistent parenting are at higher risk of abusing their children.⁹ Parental characteristics culminating in poor parent-child relationships further increase the risk of child maltreatment.⁷

Parental Psychological Functioning

Parents with mental health concerns, such as depression, anxiety, and high levels of stress, are at risk for maltreating their children.⁷ They may have difficulty adhering to appropriate parenting practices and coping with childcare.⁹ Parents who abuse substances may have altered judgment and mentation caring for their child.

Risk factors, or combination of factors, that may lead to mental health concerns also increase risk for child abuse, including

- Intimate partner violence
- A parent with a history of being abused as a child¹
- A history of other trauma
- Social issues, such as poverty and neighborhood violence

Societal Factors Affecting Parenting

Structural issues can lead to compromised functioning of parents and cause increased family stress. These may include

- Homelessness and lack of housing assistance⁹
- Living in high-poverty communities^{5,8}
- Living in violent neighborhoods
- Food insecurity
- Parental social isolation^{5,8}
- Limited access to services that promote parental mental and physical well-being

Difficult to Care for Children

Children with developmental and/or physical disabilities, behavioral issues, and chronic medical conditions are at increased risk for maltreatment.⁹ Children with such characteristics often require higher levels of care and nurturing and are challenging for even the most experienced parent. For a parent with inadequate parenting skills or other risk factors, parenting a child with a disability may be extremely stressful.

Other Risk Factors

Children younger than 3 years of age are especially vulnerable.^{5,8} One-third of children who are maltreated are between the ages of 0 and 3 years, with most victims being younger than 1 year of age.¹⁰ An important risk includes having a nonbiologically related adult in the home, because these adults are often the perpetrators of maltreatment.⁹ However, nonbiologically related adults are not always the abuser. Children may be abused by any caregiver.

ABUSIVE HEAD TRAUMA

Abusive head trauma (AHT), formerly referred to as shaken baby syndrome, describes the signs and symptoms resulting from nonaccidental shaking of the head and neck, impacting of the head and neck area, or a combination of acceleration/deceleration injury and impact.

Infant crying is a common trigger for AHT. This association is based on age-specific curves for AHT that have similar onset and patterns as age-specific curves for crying. Crying usually increases during the first month of life, peaks in the second month, and

decreases by the fourth month.¹¹ The incidence of AHT peaks between 2.5 to 4 months, with the first episode estimated to occur around 6 weeks of age.^{12,13}

Parents commonly ask primary care providers about the causes of infant crying and how to manage it.¹³ An infant with prolonged and inconsolable episodes of crying is a source of anger, frustration, powerlessness, and stress.¹¹ These feelings may be overwhelming and lead to caregiver loss of control and violence toward the infant.¹³

Parents may not know an infant's brain is fragile or understand the consequences of shaking a baby. In one study, 25% to 50% of future parents did not know shaking a baby could lead to brain injury or death.¹⁴ Low-cost prevention programs provided by maternity nurses before newborn discharge demonstrated promising results.¹⁵ Later data using a larger cohort indicated that, despite retained self-reported parental knowledge, head trauma hospitalization rates were not reduced.¹⁶

PREVENTION STRATEGIES

Successful interventions not only reduce risk factors but also promote protective factors. Families with good social supports have lower rates of physical abuse and are more likely to use nonphysical disciplinary strategies.^{9,17} A parent's sense of self-efficacy and competence enables better coping strategies and reduces the adverse effects of risk factors. For example, parents who can secure resources and services for their family's particular needs, such as services for a developmentally delayed child, are much less likely to abuse.^{9,18} Similarly, children with high self-esteem who have the supportive involvement of an adult or who are involved in extracurricular activities, may be less likely to suffer the negative consequences of maltreatment.^{9,19}

Education and support are important prevention strategies.⁷ Some strategies may be implemented through local community resources, such as home visiting or referral to a parenting program, and others may involve direct provider-to-family interaction.

Parenting programs are one of the most successful interventions for reducing child maltreatment in those with identified risk factors.⁷ Evidence has shown that parenting programs are successful in reducing the prevalence of reports of child maltreatment, preventing its recurrence, and reducing the risk factors that impair healthy functioning.⁷

Medical providers may need to develop skills in identifying risks for maltreatment and preparation for offering prevention strategies. The Safe Environment for Every Kid (SEEK) model has been reported to enable practitioners to identify and help address psychosocial problems (ie, parental depression, substance abuse, major stress, intimate partner violence, harsh punishment, and food insecurity) associated with neglect.²⁰ Results of randomized controlled trials have demonstrated promising outcomes regarding the efficacy of the SEEK model.²¹ For example, in an urban pediatric resident clinic, SEEK resulted in significantly lower rates of child maltreatment in all outcome measures: decreased child protective services (CPS) reports, problems related to neglect, nonadherence to medical care, delayed immunizations, and rates of severe/very severe physical punishment.¹

Selecting and Implementing Effective Prevention Resources

To offer comprehensive and useful local resources for families, providers must assess the effectiveness of local programs. Referrals may be influenced by

- The effectiveness of programs based on local funding, staffing, needs, and other concerns
- The program's theory (eg, interventions for parenting skills vs intervention for parents with mental health concerns)

- Form of abuse, such as head trauma prevention, support for families at risk for neglecting children, child sexual abuse (CSA) prevention programs, and others

The clearinghouse model is used to help evaluate effectiveness and inform decision-makers about validated evidence-based practices of prevention programs. Clearinghouses use simple formats to summarize empirical evidence supporting program efficacy. The California Evidence-Based Clearinghouse for Child Welfare (CEBC) offers a database of child welfare-related programs with a rating system, descriptions, and resources, including a guide to program selection and implementation. CEBC includes topic areas within primary and secondary prevention ([Table 1](#)).²² Programs that address bullying, cyberbullying, harassment, bystander actions, consent, gender, and social norms education have the potential to reduce the public health challenge of CSA.²³

Parenting Programs

Most home visiting and parent education programs successfully reduce child maltreatment.^{7,24} Components of data-based parenting programs include attachment theory, learning theory, and social learning theory principles to change parenting attitudes, practices, skills, reduction of parent-child conflict, reduction of parenting stress, improvement of psychosocial functioning, improvement of daily dynamics, and reduction of child behavioral problems.²⁵ Chen and Chan⁷ found that programs produced positive effects in low-, middle-, and high-income countries and were effective in reducing child maltreatment when applied as primary, secondary, and tertiary maltreatment interventions. The risk of maltreatment decreased after interventions. Fathers did not gain as much from parenting programs as mothers and programs may need modification for fathers. The authors also found that parenting programs started on or before the prenatal period were helpful in preventing maltreatment from ever occurring.⁷

Home visiting through nurse-family partnerships²⁴ directed toward high-risk populations, such as adolescent parents, have focused on interventions that improve neurodevelopmental, cognitive, and behavioral functioning of the child using theories of human ecology, self-efficacy, and human attachment. Nurse-family partnership programs are offered in 41 states and local resources are identified through the national Web site.²⁶

The AAP has developed prevention programs for clinical settings including Connected Kids: Safe, Strong, Secure, a resiliency-based educational program for parents and providers focused on managing challenging developmental stages, providing effective discipline, and other topics. The effectiveness of this program has not yet been evaluated.²⁷ Practicing Safety is another AAP-developed program that helps providers screen for and address maternal depression. It uses anticipatory guidance to help parents cope with challenging developmental stages, such as infant crying, colic, and toilet training.²⁸ Changes in practice behavior have been demonstrated, but effects on child maltreatment have not been examined.⁹

Other prevention education may include

- Staying Positive While Parenting, a free, online resource, is available through the New York State Department Health grant-funded program for child abuse medical providers. These handouts were developed to help parents manage their own emotions when facing children's challenging developmental stages.²⁹
- The Centers for Disease Control and Prevention offers further guidance regarding violence prevention on their Web site.³⁰
- All child caregivers, athletic coaches, and other adults who may have independent access to children may pose risks to any child. Steps to choosing

Table 1
Programs in primary and secondary prevention of child abuse and neglect

Child Welfare Related Program	Type of Prevention	Target Audience	CEBC Rating ^a	Program Information
Nurse-Family Partnership	Primary	Home visits for first-time low-income mothers with children from pregnancy through second birthday	1; well-supported by research evidence	https://www.cebc4cw.org/program/nurse-family-partnership/
Safe Environment for Every Kid	Primary	Families with children to age 5	1; well-supported by research evidence	https://www.cebc4cw.org/program/safe-environment-for-every-kid-see-model/
Triple P Positive Parenting Program System	Primary	For parents and caregivers of children to age 16	2; supported by research evidence	https://www.cebc4cw.org/program/triple-p-positive-parenting-program-system/
ACT Raising Safe Kids	Primary	Parents and caregivers of children birth to age 10	3; promising research	https://www.cebc4cw.org/program/act-raising-safe-kids/
"Who Do you Tell?"	Primary	Children kindergarten to grade 6	3; promising research	https://www.cebc4cw.org/program/who-do-you-tell/
Body Safety Training Workbook	Primary	Children ages 3–8 by their parents and teachers	3; promising research	https://www.cebc4cw.org/program/body-safety-training-workbook/
Parents as Teachers	Primary	Parents with children up to age 5	3; promising research	https://www.cebc4cw.org/program/parents-as-teachers/
Period of Purple Crying	Primary	Parents of infants up to 5 mo of age	3; promising research	https://www.cebc4cw.org/program/period-of-purple-crying/
The Safe Child Program	Primary	Children ages 3–9	3; promising research	https://www.cebc4cw.org/program/the-safe-child-program/
Safe Touches	Primary	Children in kindergarten to third grade	3; promising research	https://www.cebc4cw.org/program/safe-touches/

Stewards of Children	Primary	Staff and volunteers of schools or youth serving organizations, parents/ caregivers, concerned adults	3; promising research	https://www.cebc4cw.org/program/stewards-of-children/
Strong Communities for Children	Primary	Entire communities	3; promising research	https://www.cebc4cw.org/program/strong-communities-for-children/
Circles of Safety	Primary	Youth-serving organizations	Not able to be rated	https://www.cebc4cw.org/program/circles-of-safety/
Family Hui	Primary	Parents of children up to age 5	Not able to be rated	https://www.cebc4cw.org/program/family-hui/
The Happiest Baby	Primary	New parents, grandparents, teachers and health care professionals	Not able to be rated	https://www.cebc4cw.org/program/the-happiest-baby/
MBF Child/Teen Safety Matters	Primary	Children in grades kindergarten to 12, parents, and school personnel	Not able to be rated	https://www.cebc4cw.org/program/mbf-child-safety-matters/ https://www.cebc4cw.org/program/mbf-teen-safety-matters/
Safe Families for Children	Primary	Parents in crisis and their children up to 18 y of age who need temporary place to live	Not able to be rated	https://www.cebc4cw.org/program/safe-families-for-children-sffc/
The Incredible Years	Secondary	Parents, teachers, and children ages 4–8; versions for high-risk populations	1; well-supported by research evidence	https://www.cebc4cw.org/program/the-incredible-years/
Safe Environment for Every Kid	Secondary	Families with children to age 5 who have risk factors for abuse, such as parental depression or substance abuse	2; supported by research evidence	https://www.cebc4cw.org/program/safe-environment-for-every-kid-see-model/

(continued on next page)

Table 1 (continued)				
Child Welfare Related Program	Type of Prevention	Target Audience	CEBC Rating^a	Program Information
SafeCare	Secondary	In-home parent training program for at-risk or parents with history of abuse or neglect	2; supported by research evidence	https://www.cebc4cw.org/program/safecare/
C.A.R.E.S (Coordination, Advocacy, Resources, Education and Support)	Secondary	For families at risk for abuse or neglect; parents/caregivers with children up to age 17	3; promising research	https://www.cebc4cw.org/program/c-a-r-e-s-coordination-advocacy-resources-education-and-support/
CICC's Effective Black Parenting Program	Secondary	African-American families at risk for child maltreatment with children up to age 17	3; promising research	https://www.cebc4cw.org/program/effective-black-parenting-program/
Combined Parent-Child Cognitive-Behavioral Therapy	Secondary	For families at risk of physical abuse with children ages 3–17	3; promising research	https://www.cebc4cw.org/program/combined-parent-child-cognitive-behavioral-therapy-cpc-cbt/
Exchange Parent Aide	Secondary	For families at risk with at least one child up to age 12	3; promising research	https://www.cebc4cw.org/program/exchange-parent-aide/
Family Connections	Secondary	For families at risk, parents/ caregivers and children up to age 17	3; promising research	https://www.cebc4cw.org/program/family-connections/
Nurturing Parenting Programs for Parents and their School aged Children 5–12 y	Secondary	Families who have been reported to the child welfare system; parents and their school-aged children 5–12 y	3; promising research	https://www.cebc4cw.org/program/nurturing-parenting-program-for-parents-and-their-school-age-children-5-to-12-years/
Step-by-Step Parenting Program	Secondary	Parents with learning differences whose children are at risk of neglect	3; promising research	https://www.cebc4cw.org/program/step-by-step-parenting-program/

CICC's Los Ninos Bien Educados	Secondary	Parents of Latino descent who are raising children age 2–12 in the United States	Not able to be rated	https://www.cebc4cw.org/program/los-ninos-bien-educados/
CICC's New Confident Parenting Program	Secondary	Parents of children 2–12 y old, who are experiencing behavior or emotional problems	Not able to be rated	https://www.cebc4cw.org/program/new-confident-parenting-program/
Love and Logic	Secondary	Parents, grandparents, teachers, and other caregivers of children	Not able to be rated	https://www.cebc4cw.org/program/love-and-logic/
Make Parenting A Pleasure 2nd Edition	Secondary	At-risk, highly stressed families with children up to age 8	Not able to be rated	https://www.cebc4cw.org/program/make-parenting-a-pleasure/
Nurturing Parenting Programs for Parents and their Infants, Toddlers, and Preschoolers	Secondary	Families who have been reported to the child welfare system; parents and their infants, toddlers, and preschoolers	Not able to be rated	https://www.cebc4cw.org/program/nurturing-parenting-program-for-parents-and-their-infants-toddlers-and-preschoolers/
Steps Toward Effective, Enjoyable Parenting	Secondary	Home visits and group sessions for first-time, low-income mothers of children up to age 2	Not able to be rated	https://www.cebc4cw.org/program/steps-to-effective-enjoyable-parenting/

^a This scale is a 1 to 5 rating of the strength of the research evidence supporting a practice or program. A scientific rating of 1 represents a practice with the strongest research evidence and a 5 represents a concerning practice that seems to pose substantial risk to children and families. Some programs do not currently have strong enough research evidence to be rated on the Scientific Rating Scale and are classified as “not able to be rated.”

Data from California Evidence-Based Clearinghouse for Child Welfare (CEBC). Available at: <http://www.cebc4cw.org>. Accessed June 6, 2019.

appropriate caregivers can be addressed with parents. For example, parents of infants can be given information from the National Center on Shaken Baby Syndrome,³¹ including coursework their babysitter can complete. Parents of older children and teenagers may benefit from reading a guide produced by the Center for Behavioral Intervention in Beaverton Oregon, Protecting Your Children: Advice from Child Molesters.³²

- Prevention of CSA is largely school-based and involves the children themselves.^{33,34}

Prevention of Abuse Recurrence

Suspicion and identification of child abuse includes recognition of risk factors and physical findings. Many signs of physical abuse may be detected on careful inspection of an undressed child. Health care professionals must perform comprehensive physical examinations of children, including examination of the scalp, skin, oropharynx, and genitalia to identify potential injuries.³⁵ Comprehensive examinations allow for detection of injuries, such as bruises in unusual locations based on a child's developmental age. It is helpful to recall, "those who don't bruise, rarely bruise."³⁶

TEN-4 FACES is a clinical decision tool for bruises that may be helpful to health care professionals.³⁷ This tool predicts physical abuse (96% sensitivity and 87% specificity) with bruising anywhere on a child less than 4 months old or bruising in a child less than 4 years old on the torso, ears, neck, frenulum, angle of jaw, cheek, or eyelid or subconjunctival hemorrhage. A comprehensive screening instrument for all forms of child abuse is needed and should be a research priority.

Sentinel injuries

The term "sentinel injury" is defined by many child abuse pediatricians as a medically minor injury suspicious for abuse, and therefore, with potential for forensic significance.³⁸ Minor abusive injuries in precruising infants herald the possibility of more severe future physical abuse.^{39,40} Bruising is the most common type of sentinel injury (80%) followed by intraoral injury (11%) and fracture (7%).³⁷

Sheets and coworkers³⁸ reported 27.5% of children determined to have been abused had a previously documented sentinel injury. Health care professionals were aware of a sentinel injury in 42% of abused children. In some cases, providers suspected abuse and reported to the appropriate investigative authorities, yet the child was not protected for a variety of reasons and went on to sustain additional injuries. In other cases, the provider noted the finding, but did not suspect abuse, or the provider stated the injury was consistent with an accidental or self-inflicted mechanism.³⁸ These data suggest the need for education of health care professionals about significance of sentinel injuries.

Occult and Missed Abuse Injuries

AHT and abusive fractures are missed 30% and 20% of the time, respectively.^{39,40} Missed abuse puts children at risk for future, escalating episodes of abuse.^{39–41} Radiologic studies, such as skeletal survey and neuroimaging, diagnostic testing, such as hepatic transaminases, and a dilated ophthalmologic examination can identify occult injuries. Identification of occult injuries can increase the rate of abuse recognition.^{39–41} Appropriate and timely diagnosis of abuse improves safety planning by investigative workers, such as Department of Social Services and law enforcement. Despite available screening guidelines for occult injuries, there is significant variability in evaluations for child abuse among hospitals and individual health care

professionals.⁴¹ Use of protocols for children with sentinel injuries is expected to reduce missed abuse.⁴²

Risk of Morbidity and Mortality

Physical abuse may lead to death immediately following the injuries or many years or decades later, related to complications from abuse. At least 1500 children die from child maltreatment in the United States every year.⁴³ In 2012, Congress passed the Protect Our Kids Act and created the Commission to Eliminate Child Abuse and Neglect Fatalities (CECANF) to address this issue. Berger and colleagues⁴³ published a perspective article on this in 2015, and in 2016 CECANF published its final report, *Within Our Reach: A National Strategy to Eliminate Child Abuse and Neglect Fatalities*.⁴⁴ The following list highlights key recommendations:

- Compared with other age groups, abused infants and toddlers are at high risk for fatality.
- A call to a child protection hotline predicts later abuse or neglect fatalities, regardless of the disposition. Screening out calls may miss children who may be at a high risk.
- Health care and public health agency professionals' involvement is vital to children's safety. Interagency coordination is critical.
- The importance of child protection workers' access to real-time information about families is of great value for the protection of children. Primary care providers often have access to much of this information.
- Accurate data allow a better understanding of what works and what does not.
- The Nurse-Family Partnership program has saved lives.⁴⁵

Reporting and Treating Child Maltreatment

Forty-eight states, the District of Columbia, American Samoa, Guam, the Northern Mariana Islands, Puerto Rico, and the Virgin Islands designate by law all physicians, nurses, and other health care workers who have frequent contact with children as mandated reporters. Approximately 18 states and Puerto Rico require any person, regardless of profession, to report abuse or neglect if suspected.⁴⁶ Health care providers must be aware of the laws in their region.

To protect maltreated children, reporting laws in most states restrict a mandated reporter's "privileged communications," the right to maintain confidential communications between professionals and their patients. Although the mandated reporter may choose to remain anonymous in most states, it is helpful to the investigation to know the reporter's identity. The identity of the reporter is not disclosed to the alleged perpetrator in most states. The 2016 factsheet, *Mandatory Reporters of Child Abuse and Neglect*, by the Child Welfare Information Gateway provides the statutes for specific states or territories.⁴⁶

In January 2019 the bipartisan Victims of Child Abuse Reauthorization Act (VOCARA) was signed into law. This law, originally known as the Victims of Child Abuse Act of 1990, initially authorized funding to Children's Advocacy Centers to help victims of child abuse. The 2019 version of this law contains language that further supports the role of health care professionals who provide primary care to children. For many years, mandated reporters have had immunity from legal action if a report was made "in good faith." VOCARA provides the same legal protections to health care professionals who assist with investigations of cases of suspected child maltreatment (eg, a pediatric hospitalist who admits a suspected victim of child abuse and subsequently orders radiographs). This law addresses the trend of pediatricians,

and in particular child abuse pediatricians, who are confronted with legal action for their roles in a child's medical evaluation for abuse.⁴⁷

Prevention of Child Sexual Abuse

A multifaceted prevention approach is necessary to address the public health challenge of CSA. Traditionally, tertiary prevention approaches have been used for treatment after an offense occurred, both for survivors of CSA to reduce long-term effects of CSA and its recurrence and for offending individuals. Interventions are needed at primary and secondary prevention levels to treat and support those at risk of sexually offending.⁴⁸ Effective primary prevention aimed at potential victims reaches children across all ages beginning with early development of social emotional health. In addition, it tasks adults with developing methods to change social norms and behaviors and to intervene before abuse occurs. Secondary prevention programs target populations at-risk, such as those homeless or under economic stress, children of parents with a mental illness or drug dependency, and LGBTQ+ youth.

Effective prevention efforts require recognition of risk and protective factors at the level of the individual, community, or a society (Table 2). Clinical implications of systemic risk factors for the child, parent, family, and community include developing risk assessment strategies and care needs assessments, and to identify those in need of preventive care and what care may be needed.⁴⁹

Expectations for Medical Professionals

Pediatricians have long reported giving inconsistent anticipatory guidance to patients and families about sexual abuse, are uncomfortable discussing sexual issues, were not trained to recognize red flags for sexual abuse, and do not have a consistent approach to cases of suspected abuse.⁵⁰ Medical professionals unknowingly see pediatric patients affected by sexual abuse and must be familiar with its definition, including recognition of healthy, concerning, or abusive behaviors, and an understanding of the unique dynamics related to CSA, such as grooming evolution and disclosures.

Prevention before abuse occurs is ideal. Identification of CSA may offer protection from revictimization. Limited CSA screening tools either evaluate behavior and identify symptoms, or identify at-risk youth.

Definition of Child Sexual Abuse

Prevention of CSA requires an understanding of the definition and dynamics of sexual abuse. As defined by the 1999 World Health Organization Consultation on Child Abuse Prevention, "child sexual abuse is the involvement of a child in sexual activity that he or she does not fully comprehend, is unable to give informed consent to, or for which the child is not developmentally prepared and cannot give consent, or that violates the laws or social taboos of society. Child sexual abuse is evidenced by this activity between a child and an adult or another child who by age or development is in a relationship of responsibility, trust, or power, with the activity being intended to gratify or satisfy the needs of the other person. This may include, but is not limited to: the inducement or coercion of a child to engage in any unlawful sexual activity; the exploitative use of a child in prostitution or other unlawful sexual practices; or the exploitative use of children in pornographic performance and materials."⁵¹

Grooming

Understanding the dynamics of CSA helps to improve health care provider identification of suspected cases and enable intervention before escalation of sexual abuse

Table 2
Risk and protective factors for CSA victimization

	Individual/Interpersonal/Family	Community/ Organization	Society
Risk factors	Prior CSA victimization of child and/or siblings, other family members	Poverty	Social climate that tolerates sexual abuse
	Prior or concurrent forms of child abuse in the home	Youth serving organization setting	Insufficient laws and policies
	Parental problems, such as domestic violence, nonconnectedness, nonnuclear family structure, in foster care	“Virtual” settings, cyber-interactions	
	Parental problems, such as domestic violence, nonconnectedness, nonnuclear family structure, in foster care	Lack of institutional support from police and judicial system	
	Poor parent-child relationship, child isolation Female gender Situational factors		
Protective factors and resilience	Presence of caring and supportive adult	Structured, positive school environment	Access to affordable quality health care, including mental health, and social welfare services
	Parental monitoring	Involvement in religious community	Adequate laws and policies
	Social support and connectedness	Involvement in extracurricular activities or hobbies	Research and training programs, public awareness, and education
	Attributes of positive disposition or temperament of child, including strong cognitive ability, internal locus of control, ego control, high self-esteem, spirituality		

Data from Refs. ^{2,49,61}

behaviors. This includes knowledge of grooming, which builds trust by a perpetrator, who may be a known and trusted caregiver.⁵²

Grooming behaviors may include

- Subtle approaches designed to build relationships
- Catering to a child’s interests
- Created opportunities to be alone with a child or children
- Special privileges/gifts/favoritism

Disclosure

Children may not disclose an act because of pressure by the perpetrator, concern of consequences, fear of not being believed, lack of opportunity, or a lack of

understanding of CSA. A high percentage of sexually abused children hold disclosure until adulthood; therefore, health care professionals can expect to encounter children and adolescents who are withholding disclosure.⁵³ Implementation of organizational-level protocols ensures quick and supportive responses to disclosure and connects the child to professional services to keep the child safe and facilitate recovery from trauma.

Trafficking

Child victims of labor and sex trafficking are often difficult to identify and have complex medical and mental health needs. National legislation, Stop, Observe, Ask and Respond (SOAR) to Health and Wellness Act, directs the US Department of Health & Human Services to provide health care professionals with training on how to identify and appropriately treat human trafficking victims. There is a need for standardization to ensure accuracy of information, use of trauma-informed and patient-centered care, and consistent messaging. A six-item questionnaire has been shown to be effective in identifying commercial sexual exploitation of children and child sex trafficking victims among a high-risk adolescent population.⁵⁴

Health systems can meet the health care needs of victims by^{55–57}

- Providing trafficking training to educate practitioners
- Documenting suspected and confirmed trafficking using International Classification of Diseases-10-US codes
- Establishing an internal response protocol with trauma-informed/patient-centered care
- Implementing an information and referral network for local services offered
- Conspicuously posting the National Human Trafficking Hotline
- Investing in community advocacy, including funding and/or time toward anti-human trafficking initiatives

Prevention of Peer Victimization and Social Media Risks

Peer victimization and child abuse via social media have become areas of grave concern.^{58–60} Bullying of children by peers is known to cause mental health, physical, and long-term health consequences, including depression, suicidal ideation, and/or suicide attempts. There is a strong association of cyberbullying with suicide, above and beyond the effects of traditional bullying. Cyberbullying, defined by Kowalski and coworkers,⁵⁸ is “the use of electronic communication technologies to bully others.”

Social media risks go beyond typical face-to-face bullying, including amplifying and creating a widespread pervasive abusive environment. Tactics can include harassment, humiliation, and other cruel intents. The added risks associated with activated global positioning systems (GPS) satellite services enables perpetrators to find vulnerable children who use GPS applications (eg, Pokémon Go, or dating applications, such as Tinder). However, GPS has many positive uses, including enabling parents to track their children’s whereabouts. Likewise, social media can provide important educational and emotional support for children and their parents. Prevention of cyberbullying and the reduction of the risks and harm of social media to children and adolescents requires oversight and vigilance by parents and caregivers.

To identify risks for bullying and cyberbullying, the provider can incorporate some broad questions and discussions, such as asking about experiences with social media, use and types of smart-phone applications, and about observations regarding peers who may be having problems online.⁵⁹

It has been traditionally believed that schools should be responsible for implementing bullying interventions. However, cyberbullying does not just take place in schools and online activities can take place anywhere. The health effects are significant. Hutson⁶⁰ recommends that providers use evidence-based, effective programming commonly implemented by schools, such as teaching communication/social skills, empathy training, coping skills, and education on digital citizenship. This is provided through referral for mental health support, or directly by the health care provider. Parents and children should be part of interventions.⁶⁰

SUMMARY

Opportunities to prevent child maltreatment and abuse can be integrated into the pediatric health care visit. Providers should take a broad view of the social, emotional, structural, and family context to identify risks for abuse. Key areas include the public health framework, including support systems that are available to parents. It is important to recognize risk factors for abuse and potential examination findings of physical abuse (especially sentinel injuries) to intervene and prevent further abuse. Sexual abuse prevention is challenging and recognition of signs, including signs of trafficking and peer victimization, requires vigilance. Mandated reporters are protected from liability by federal law as long as they make a report "in good faith." Expansion of this law provides further protection for health care professionals who evaluate and treat suspected child abuse victims.

DISCLOSURE

The authors have nothing to disclose.

REFERENCES

1. Dubowitz H, Feigelman S, Lane W, et al. Pediatric primary care to help prevent child maltreatment: the safe environment for every kid (SEEK) model. *Pediatrics* 2009;123(3):858–64.
2. Flaherty EG, Stirling J. The pediatrician's role in child maltreatment prevention. *Pediatrics* 2010;126(4):833.
3. U.S. Preventive Services Task Force. Interventions to Prevent Child Maltreatment: Recommendation Statement. *Am Fam Physician* 2019;100(2):110–2.
4. U.S. Department of Health & Human Services. Administration for Children & Families, Children's Bureau, Child Welfare Information Gateway. Framework for Prevention of Child Maltreatment. 2017. Available at: <https://www.childwelfare.gov/topics/preventing/overview/framework/>. Accessed April 25, 2019.
5. Harden BJ, Buhler A, Parra LJ. Maltreatment in infancy: a developmental perspective on prevention and intervention. *Trauma Violence Abuse* 2016;17(4):366–86.
6. Flaherty EG, Legano L, Idzerda S. Ongoing pediatric health care for the child who has been maltreated. *Pediatrics* 2019;143(4):e20190284.
7. Chen M, Chan KL. Effects of parenting programs on child maltreatment prevention: a meta-analysis. *Trauma Violence Abuse* 2016;17(1):88–104.
8. Pelton LH. The continuing role of maternal factors in child maltreatment and placement. *Child Abuse Negl* 2015;41:30–9.
9. Lane WG. Prevention of child maltreatment. *Pediatr Clin North Am* 2014;61(5):873–88.

10. U.S. Department of Health and Human Services, Administration on Children, Youth and Families. Child maltreatment 2017. Available at: <https://www.acf.hhs.gov/sites/default/files/cb/cm2017.pdf>. Accessed April 25, 2019.
11. Barr RG, Barr M, Fujiwara T, et al. Do educational materials change knowledge and behaviour about crying and shaken baby syndrome? A randomized controlled trial. *CMAJ* 2009;180(7):727–33.
12. Barr RG, Trent RB, Cross J. Age-related incidence curve of hospitalized shaken baby syndrome cases: convergent evidence for crying as a trigger to shaking. *Child Abuse Negl* 2006;30(1):7–16.
13. Goulet C, Frappier JY, Fortin S, et al. Development and evaluation of a shaken baby syndrome prevention program. *J Obstet Gynecol Neonatal Nurs* 2009;38(1):7–21.
14. Showers J. "Don't shake the baby": the effectiveness of a prevention program. *Child Abuse Negl* 1992;16(1):11–8.
15. Altman RL, Canter J, Patrick PA, et al. Parent education by maternity nurses and prevention of abusive head trauma. *Pediatrics* 2011;128(5):e1164–72.
16. Dias MS, Rottmund CM, Cappos KM, et al. Association of a postnatal parent education program for abusive head trauma with subsequent pediatric abusive head trauma hospitalization rates. *JAMA Pediatr* 2017;171(3):223–9.
17. Lyons SJ, Henly JR, Schuerman JR. Informal support in maltreating families: its effect on parenting practices. *Child Youth Serv Rev* 2005;27:21–38.
18. Coleman PK, Karraker KH. Self-efficacy and parenting quality: findings and future applications. *Developmental Rev* 1998;18:47–85.
19. Runyan DK, Hunter WM, Socolar RR, et al. Children who prosper in unfavorable environments: the relationship to social capital. *Pediatrics* 1998;101(1 Pt 1):12–8.
20. Dubowitz H. The safe environment for every kid model: promotion of children's health, development, and safety, and prevention of child neglect. *Pediatr Ann* 2014;43(11):e271–7.
21. Dubowitz H, Lane WG, Semiatin JN, et al. The safe environment for every kid model: impact on pediatric primary care professionals. *Pediatrics* 2011;127(4):e962–70.
22. California Department of Social Services, Office of Child Abuse Prevention. The California evidence-based clearinghouse for child welfare. 2019. Available at: <https://www.cebc4cw.org/>. Accessed June 6, 2019.
23. Breiner H, Ford M, Gadsden VL, editors. Parenting matters: supporting parents of children ages 0-8. Washington, DC: National Academy of Sciences; 2016.
24. Olds DL. Preventing child maltreatment and crime with prenatal and infancy support of parents: the nurse-family partnership. *J Scand Stud Criminol Crime Prev* 2008;9(S1):2–24.
25. Vlahovicova K, Melendez-Torres GJ, Leijten P, et al. Parenting programs for the prevention of child physical abuse recurrence: a systematic review and meta-analysis. *Clin Child Fam Psychol Rev* 2017;20(3):351–65.
26. Nurse-Family Partnership. Nurse-Family Partnership Web site. 2019. Available at: <https://www.nursefamilypartnership.org/first-time-moms/>. Accessed June 20, 2019.
27. American Academy of Pediatrics. Connected Kids: Safe, Strong, Secure. 2019. Available at: <https://www.aap.org/en-us/advocacy-and-policy/aap-health-initiatives/Pages/Connected-Kids.aspx>. Accessed June 17, 2019.
28. American Academy of Pediatrics. Practicing Safety Program. 2019. Available at: <https://www.aap.org/en-us/advocacy-and-policy/aap-health-initiatives/practicing-safety/Pages/Phase-I.aspx>. Accessed June 6, 2019.

29. Child Abuse Medical Provider Program. Staying positive while parenting. CHAMP Program Web site. 2019. Available at: <http://champprogram.com/pdf/Staying-Positive-While-Parenting-pamphlet.pdf>. Accessed June 19, 2019.
30. Centers for Disease Control and Prevention. Technical packages for violence prevention: using evidence-based strategies in your violence prevention efforts. 2019. Available at: <https://www.cdc.gov/violenceprevention/pub/technical-packages.html>. Accessed June 22, 2019.
31. National Center on Shaken Baby Syndrome. Available courses. 2019. Available at: <https://training.dontshake.org/courses>. Accessed June 18, 2019.
32. The Center for Behavioral Intervention in Beaverton, Oregon. Protecting your children: advice from child molesters. Vermont Official State Website: Agency of Human Services Department for Children and Families Web site. 2019. Available at: <https://dcf.vermont.gov/sites/dcf/files/Prevention/docs/Protecting-Children.pdf>. Accessed June 15, 2019.
33. Black DA, Heyman RE, Smith Slep AM. Risk factors for child sexual abuse. *Aggress Violent Behav* 2001;6(2):203–29.
34. Davis MK, Gidycz CA. Child sexual abuse prevention programs: a meta-analysis. *J Clin Child Psychol* 2000;29(2):257–65.
35. Christian CW. The evaluation of suspected child physical abuse. *Pediatrics* 2015; 135(5):e1337–54.
36. Sugar NF, Taylor JA, Feldman KW. Bruises in infants and toddlers: those who don't bruise rarely bruise. Puget Sound Pediatric Research Network. *Arch Pediatr Adolesc Med* 1999;153(4):399–403.
37. Clyde Pierce M, Kaczor K, Aldridge A, et al. Bruising characteristics discriminating physical child abuse from accidental trauma. *Pediatrics* 2010;125(1): 67–74.
38. Sheets LK, Leach ME, Koszewski IJ, et al. Sentinel injuries in infants evaluated for child physical abuse. *Pediatrics* 2013;131(4):701–7.
39. Jenny C, Hymel KP, Ritzen A, et al. Analysis of missed cases of abusive head trauma. *JAMA* 1999;281(7):621–6.
40. Ravichandiran N, Schuh S, Bejuk M, et al. Delayed identification of pediatric abuse-related fractures. *Pediatrics* 2010;125(1):60–6.
41. Wood JN, Feudtner C, Medina SP, et al. Variation in occult injury screening for children with suspected abuse in selected US children's hospitals. *Pediatrics* 2012;130(5):853–60.
42. Lindberg DM, Juarez-Colunga E, Wood JN, et al. Testing for abuse in children with sentinel injuries. *Pediatrics* 2015;136(5):831–8.
43. Berger RP, Sanders D, Rubin D. Commission to Eliminate Abuse and Neglect Fatalities. Pediatricians' role in preventing child maltreatment fatalities: a call to action. *Pediatrics* 2015;136(5):825–7.
44. U.S. Department of Health & Human Services. Administration for children & families, children's bureau, commission to eliminate child abuse and neglect fatalities. Within our reach: a national strategy to eliminate child abuse and neglect fatalities. Washington, DC: Government Printing Office; 2016. Available at: <http://www.acf.hhs.gov/programs/cb/resource/cecانf-final-report>, 2016. Accessed on March 19, 2020.
45. Child Welfare League of America. Commission releases final report on child fatalities. 2016. Available at: <https://www.cwla.org/commission-releases-final-report-on-child-fatalities/>. Accessed June 20, 2019.
46. U.S. Department of Health & Human Services. Administration for Children & Families, Children's Bureau, Child Welfare Information Gateway. Mandatory reporters

- of child abuse and neglect. 2017. Available at: <https://www.childwelfare.gov/pubpdfs/manda.pdf>. Accessed April 25, 2019.
47. Library of the Congress. Victims of Child Abuse Act Reauthorization Act of 2018. 2018. Available at: <https://www.congress.gov/bill/115th-congress/senate-bill/2961/all-info>. Accessed July 24, 2019.
 48. Knack N, Winder B, Murphy L, et al. Primary and secondary prevention of child sexual abuse. *Int Rev Psychiatry* 2019;31(2):181–94.
 49. Assink M, van der Put CE, Meeuwssen MWCM, et al. Risk factors for child sexual abuse victimization: a meta-analytic review. *Psychol Bull* 2019;145(5):459–89.
 50. Leder MR, Emans J, Hafler JP, et al. Addressing sexual abuse in the primary care setting. *Pediatrics* 1999;104:270–5.
 51. World Health Organization. Violence and Injury Prevention Team & Global Forum for Health Research, Consultation on Child Abuse Prevention. Report of the Consultation on Child Abuse Prevention; 1999. Available at: <https://apps.who.int/iris/handle/10665/65900>. Accessed on March 19, 2020.
 52. World Health Organization. Chapter 7, Child Sexual Abuse in Guidelines for medico-legal care of victims of sexual violence. 2003. Gender and Women's Health, Family and Community Health Injuries and Violence Prevention, Noncommunicable Diseases and Mental Health, Geneva, Switzerland.
 53. Townsend C. Child sexual abuse disclosure: what practitioners need to know. 2016. Available at: https://www.d2l.org/wp-content/uploads/2016/10/ChildSexualAbuseDisclosurePaper_20160217_v.1.pdf. Accessed June 6, 2019.
 54. Greenbaum J, Dodd M, McCracken C. A short screening tool to identify victims of child sex trafficking in the health care setting. *Pediatr Emerg Care* 2018;34(1):33–7.
 55. Greenbaum J, Stoklosa H. The healthcare response to human trafficking: a need for globally harmonized ICD codes. *PLoS Med* 2019;16(5).
 56. Greenbaum J, Crawford-Jakubiak JE. Committee on Child Abuse and Neglect. Child sex trafficking and commercial sexual exploitation: health care needs of victims. *Pediatrics* 2015;135(3):566–74.
 57. Polaris. National Human Trafficking Hotline. 2019. Available at: <https://humantrafficking.org/>. Accessed June 2, 2019.
 58. Kowalski RM, Giumetti GW, Schroeder AN, et al. Bullying in the digital age: a critical review and meta-analysis of cyberbullying research among youth. *Psychol Bull* 2014;140(4):1073–137.
 59. Englander E, Donnerstein E, Kowalski R, et al. Defining cyberbullying. *Pediatrics* 2017;140(Suppl 2):S148–51.
 60. Hutson E. Cyberbullying in adolescence: a concept analysis. *ANS Adv Nurs Sci* 2016;39(1):60–70.
 61. McKillop N. Understanding the nature and dimensions of child sexual abuse to inform its prevention. In: Bryce I, Robinson Y, Petherick W, editors. *Child abuse and neglect: forensic issues in evidence, impact and management*. Cambridge (MA): Elsevier; 2019. p. 241–59.